



Suites 4-7, 193 Minna Jane Drive
Charlottetown, PE C1E 0C4

 info@redsandspm.com

 www.redsandspm.com

 +1 782-377-7813

Ketamine Infusion Therapy for Pain and Depression: Patient Information Leaflet

What is Ketamine?

A medically approved anesthetic since 1970, now used in low-dose infusions for:

- Chronic Refractory Pain due to Complex Regional Pain Syndrome (CRPS), neuropathic pain and migraine headaches
- Treatment-Resistant Depression (TRD) when more than 2 antidepressants fail.

How It Works?

Modulates the nervous system pathways to "reset" malfunctioning signals:

- For Pain: Blocks NMDA receptors → reduces nerve sensitization + dampens inflammation
- For Depression: Stimulates synaptic regrowth → rapidly improves mood (effects in hours vs. weeks with traditional antidepressants)

Note: This use is "off-label" (not officially licensed for chronic pain and depression), but it is widely accepted in pain management practice.

Treatment Process

Pre-Infusion

- Medical evaluation
- Fasting solid foods for 6hrs, clear liquids including black coffee can be had up to 2 hours prior to your scheduled infusion time.

During IV infusion (60 minutes)

- Monitored BP/ECG and blood oxygen levels
- May experience dissociation "floating", drowsiness, dreams, nausea and anxiety.

Post-Infusion

- Recovery observation (1-2hr)
- Arrange ride home + 24hr companion
- No driving or operating machinery
- No signing any legal documents for 24hrs

Key Benefits

✅ For Chronic Pain:

- 50-70% report more than 30% pain reduction
- Reduces "pain catastrophizing" & improves daily function
- Effects can last for 3-4 weeks after each infusion

✅ For Depression

- Up to 70% show reduced suicidal thoughts after regular infusions.
- Up to 43% remission rate at 12 months (vs. 5% with standard antidepressants)

Risks & Side Effects

Most are short-lived and mild (resolve in 15min–48hrs).

Common (10-50%)

You could be given additional medicines to reduce some of the side effects like anxiety and nausea/vomiting.

- Dissociation/dizziness
- Nausea/vomiting
- Headache/fatigue
- BP/heart rate increase

Less Common (<10%)

- Vivid dreams
- Hallucinations
- Worsening depression (temporary)
- Anxiety/panic attacks

Rare

- Liver toxicity
- Bladder issues (high dose)
- Cognitive changes
- Mania (if bipolar)

Critical Contraindications

- Psychosis/schizophrenia history
- Uncontrolled hypertension
- Pregnancy or substance use disorder
- Dementia symptoms
- History of Stroke
- Heart failure

Long-Term Safety

- Dependence: Medical use has low addiction risk (vs. recreational abuse)
- Tolerance: Dose adjustments or breaks needed if effects diminish
- Bladder/Kidney: Only seen with frequent high-dose abuse (>1g/day)

Realistic Expectations

- 50% of chronic pain/depression patients find meaningful relief worth continuing.
- Not a cure: Works best combined with physical therapy (pain) or counseling (depression).
- Response varies: Most patients need regular infusions

Questions? Contact Your Care Team

Before/after infusion:

 782-377-7813



Info@redsandspm.com

In an emergency

→ Call 911, or visit your local emergency department

